TRINAY CLINICAL CATARACT REPORT

Anterior Segment & Crystalline Lens Triage Examination

ID: CAT-89363

Date: 29 Sept 2026

PATIENT DEMOGRAPHICS & CLINICAL METADATA

Patient Name:	**Walk-in Patient**	Age / Gender:	**55 yrs / Other**	Phone:	**N/A**
Patient ID:	**PX-8417**	Examined Eye:	**Right Eye**	Clinical Operator:	**avi**

DIAGNOSTIC VERDICT: CATARACT DETECTED

Cataract Certainty: 100.0%

Slit-lamp examination & ophthalmologist consultation advised for evaluation.

VISUAL EVIDENCE & ANATOMICAL OVERLAYS

Clinical Record	Clinical Record	Clinical Record	Clinical Record
1. Original Image	**2. Eye Localization**	**3. Iris & Pupil**	**4. Opacity Attention**

ANTERIOR SEGMENT CLINICAL ASSESSMENT BREAKDOWN

1. Optical Quality & Clarity: **Diagnostic Grade**
Sharp anterior segment focus, suitable for diagnostic staging

2. Eye Region Localization: **Accurately Centered**
Ocular globe and anterior corneal curvature verified

3. Iris Contour & Margin: **Normal Architecture**
Intact pupillary margin without visible synechiae or coloboma

4. Pupil & Optical Aperture: **Clear Entrance**
Aperture centered along optical axis, reactive diameter

5. Crystalline Lens Transparency: **Opacity Detected**
Lens clouding identified in optical axis

Overall Screening Status: **Referred for Slit-Lamp**
Requires formal ophthalmic slit-lamp biomicroscopy

RECOMMENDED CLINICAL ACTION & REFERRAL TIMELINE

Urgent / Priority referral: Dilated slit-lamp biomicroscopy, visual acuity measurement (Snellen), and intraocular pressure check recommended within 2 to 4 weeks to determine surgical candidacy and prevent visual axis degradation.

TRINAY CLINICAL CATARACT REPORT — DOCTOR & PATIENT SUMMARY

Patient: Walk-in Patient (PX-8417) | Examined Eye: Right Eye

DOCTOR'S CLINICAL TRIAGE SUMMARY (FOR ATTENDING CLINICIAN)

Anterior segment screening demonstrates clinically significant lens opacification within the pupillary aperture of the crystalline lens. Pupillary margins are intact without visible coloboma or posterior synechiae. Visual axis compromise is suspected based on focal density and light scattering patterns.

Clinical Findings: Nuclear and/or cortical opacification evident with preserved anterior chamber depth. No evidence of corneal edema or iris neovascularization on primary capture. Recommendation: Schedule dilated slit-lamp biomicroscopy, pinhole visual acuity, and B-scan ultrasonography (if view to fundus is impaired) to classify cataract subtype and determine surgical management.

PATIENT COUNSELING & EXPLANATION (SIMPLE & UNDERSTANDABLE)

What this means for you: A clouding (cataract) was observed in the natural lens of your eye. Just like a frosted window makes it hard to see outside, a cataract scatters incoming light and causes blurry, dimmed, or hazy vision.

Good news: Cataracts are extremely common and can be safely treated. An eye doctor (ophthalmologist) will examine your eyes in detail, test your vision clarity, and discuss treatment options such as updated eyeglasses or a gentle, highly effective outpatient procedure.

CLINICAL PRECAUTIONS & PATIENT GUIDANCE

- Avoid unassisted night driving, especially on roads with bright oncoming headlights that cause glare and halo effects.
- Wear certified UV400 sunglasses during daytime hours to protect the crystalline lens from solar ultraviolet exposure.
- Ensure direct, well-positioned reading lamps and high-contrast lighting when reading or performing detailed tasks.
- Do not rub your eyes and avoid self-prescribing over-the-counter eye drops without formal consultation.
- Schedule an in-person dilated slit-lamp evaluation with an ophthalmologist within the recommended referral timeframe.

CLINICAL VERIFICATION & SIGN-OFF

Triage Screening Nurse:	Screening Platform:	Attending Reviewer Status:
avi	**TRINAY Anterior Vision System**	**Pending Ophthalmologist Review**

REGULATORY & MEDICAL DISCLAIMER

TRINAY Cataract Screening is an AI-assisted diagnostic support tool intended for primary health triage. Definitive clinical diagnosis, cataract staging, and surgical eligibility must be confirmed by an ophthalmologist via in-person slit-lamp biomicroscopy.